

Patient Name: Elena Rodriguez

DOB: 11/12/1963 (Age: 62)

Sex: Female

MRN: 4455091

Encounter Info

Date of Service: 05/05/2026

Encounter Type: Office Visit - Routine Diabetic Follow-up

Provider: Dr. Jonathan Reed, MD (Family Medicine)

Chief Complaint

"Checking in on my blood sugar and reviewing my recent blood work."

History of Present Illness (HPI)

The patient is a 62-year-old female with a 10-year history of Type 2 Diabetes Mellitus, presenting for a routine 6-month follow-up. She reports that she has been fairly consistent with her Metformin and Jardiance, though she admits to "sliding a bit" on her diet over the holiday season and into the spring. She monitors her blood glucose at home 3-4 times a week; fasting readings have been ranging between 130-155 mg/dL. She denies any recent episodes of symptomatic hypoglycemia (shaking, sweating, confusion). She reports no new numbness or tingling in her feet, no changes in vision, and no significant weight changes. She is currently trying to walk for 20 minutes most evenings.

Review of Systems (ROS)

- Constitutional: Negative for fatigue or unintentional weight loss.
- Eyes: Negative for blurred vision. Last dilated eye exam was 9 months ago.
- Cardiovascular: Negative for chest pain or exertional dyspnea.
- Neurological: Negative for paresthesias in a "stocking-glove" distribution.

- Integumentary: Negative for any slow-healing sores or ulcers on the lower extremities.

Vital Signs

- BP: 134/78 mmHg (Left Arm, Seated, Large Cuff)
- HR: 72 bpm
- RR: 14 breaths/min
- Temp: 98.2 °F (Oral)
- SpO2: 98% on room air
- Wt: 188 lbs (85.3 kg)
- BMI: 31.2 (Class I Obesity)

Physical Examination

- General: Alert and oriented x3. Appears in good health, no acute distress.
- Cardiovascular: Regular rate and rhythm. No murmurs or peripheral edema.
- Respiratory: Lungs clear to auscultation bilaterally.
- Abdominal: Soft, non-tender, non-distended.
- Lower Extremities/Feet:
 - Skin is warm and dry.
 - Pedal pulses (DP and PT) are 2+ bilaterally.
 - Monofilament testing (10g): Sensation is fully intact bilaterally (10/10 points).
 - No calluses, fungal infections, or interdigital maceration noted.

Laboratories / Results

- HbA1c: 8.0% (Collected: 05/03/2026)
- Estimated Average Glucose (eAG): 183 mg/dL
- BMP: Glucose 162 mg/dL (non-fasting). Creatinine 0.85 mg/dL (eGFR >60). Electrolytes normal.
- Lipid Panel: LDL 88 mg/dL, HDL 42 mg/dL, Triglycerides 165 mg/dL.
- Urine Microalbumin/Creatinine Ratio: 18 mg/g (Normal <30).

Assessment/Plan

1. Type 2 Diabetes Mellitus without complications (E11.9): Control has slightly declined since last visit (HbA1c was 7.4% previously). Current HbA1c is 8.0%, which is at the upper limit of her individualized target (goal <7.5–8.0% given her age and duration of disease).
 - Plan: Discussed the importance of diet, specifically carbohydrate counting. Will increase Metformin ER from 1000mg to 1500mg daily to see if we can bring the A1c closer to 7.5%. Continue Jardiance 10mg daily.
2. Hyperlipidemia (E78.5): Stable on Atorvastatin 20mg. LDL remains at goal.
3. Obesity, Class I (E66.01): Encouraged increasing physical activity to 150 minutes per week. Referred to a Registered Dietitian for medical nutrition therapy.

Provider Notes

Patient was counseled today on the significance of her HbA1c reaching 8.0%. We spent considerable time discussing the "borderline" nature of this reading and the need for stricter glycemic control to prevent long-term microvascular complications. She is receptive to the medication adjustment and the referral to a nutritionist. We will re-check her HbA1c and basic metabolic panel in 3 months to assess the impact of the Metformin increase. Patient understands the signs of hypoglycemia and when to seek medical attention.